

Recommended, reimbursed — and still not completed.

A partnership brief on closing the gap between the preventive care that is recommended and the preventive care that actually gets finished — in the safety-net settings where the gap is widest.

Proven preventive services exist, but **completion** fails — colorectal-cancer screening sits below target, worst in the safety net. The Prevention Adoption Initiative is a disease-agnostic engine that finds who is overdue, removes friction, and drives completion — with an explainable, fairness-audited dashboard. I am looking for a clinic to pilot it and partners to scale it.

THE PROBLEM — ADOPTION, NOT SCIENCE

~8%

of U.S. adults 35+ receive all 15 high-priority preventive services (Borsky, *Health Affairs* 2018).

63.5% → 72.8%

CRC screening vs. the Healthy People 2030 target — barely off baseline (2023).

~44%

CRC screening in Federally Qualified Health Centers (2018 UDS) — the gap concentrates here.

THE APPROACH — FOUR PILLARS, ONE COMPLETION LOOP

Service redesign & friction reduction

Make the completed path the easiest one — defaults, mailed at-home tests, one-click scheduling.

Behaviorally-configured data infrastructure

Registries + dashboards that surface who is overdue and where patients drop out; automated reminders.

Applied AI, explainable

A proposed model layer that flags likely drop-offs and prioritizes outreach — fairness-audited, human-in-the-loop.

Implementation enablement

Training so a site can run and sustain the toolkit itself — the pillar the evidence says matters most.

Identify → **Reduce friction** → **Communicate** (literacy-tailored) → **Escalate** (community-health-worker outreach) → **Measure**, incl. diagnostic follow-up after a positive result.

THE DEMONSTRATION

A proposed 12-month colorectal-cancer completion pilot.

One Southeast Michigan FQHC below the UDS benchmark, with a FHIR-capable EHR and a quality-improvement sponsor. Evaluated with RE-AIM against the site's own baseline, pre-registering a benchmarked target above the ~3.4-point STOP CRC average, with a comparison clinic where feasible, a named post-grant sustainability path, and guaranteed diagnostic follow-up. Disease-agnostic by design — the same loop extends to cervical and breast screening, hypertension, and immunizations.

THE ASK

If your organization owns preventive-care completion, there is a way in.

Health centers / FQHCs

Host the pilot — raise a UDS/HEDIS measure you already report, inside your existing staff.

Funders / partners

Fund a cost-effective (~\$90/added screen), equity-stratified, scalable model.

Researchers

Co-design a pre-registered, publishable stepped-wedge / RE-AIM evaluation.

Md Shafaat Ali Choyon — Founder, HSREP · MPH,

CHES®

hsraep.org/initiative · Book 30 min: calendly.com/shafaat-alic/30min

LinkedIn: [/in/sacgrowthspecialist](#)

An HSREP initiative in development. All designs are illustrative and future-tense; no patient data has been collected. Operational materials will be reviewed by counsel before any launch.